

To: Steve Posnack, Principal Deputy Assistant Secretary for Technology Policy, Principal Deputy National Coordinator for Information Technology, The Office of the National Coordinator for Health IT

Re: HHS Health Sector AI RFI - Request for Information – Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

1. Introduction and Perspective

Elsevier Inc. appreciates the opportunity to respond to this Request for Information on accelerating the adoption and use of artificial intelligence (AI) in clinical care.

Elsevier is a global leader in advanced information and decision support. For over a century, we have been helping advance science and healthcare to advance human progress. We support academic and corporate research communities, doctors, nurses, future healthcare professionals and educators across 170 countries in their vital work. We do this by delivering mission-critical insights and innovative solutions that combine trusted, evidence-based scientific and medical content with cutting-edge AI technologies to help impact makers achieve better outcomes.

Our Clinical Solutions team develops **evidence-based clinical decision support (CDS)** solutions used by hospitals, integrated delivery networks (IDNs), clinicians, and care teams across the world. Our products are designed to assist clinicians at the point of care by providing timely access to trusted medical knowledge and insights from current research. By integrating into the clinical workflow, our solutions support clinicians to make informed decisions, enhancing the quality and efficiency of healthcare delivery.

Our perspective is grounded in decades of experience delivering CDS in regulated healthcare environments, maintaining **editorial independence**, clinical governance, and patient safety, while responsibly adopting new technologies such as generative AI.

2. AI in Clinical Care: Value and Scope

AI-enabled CDS tools that support **information retrieval, contextualize, -and summarization** already play a meaningful role in clinical care by:

- Improving clinician efficiency and reducing administrative and cognitive burden,
- Supporting consistency with evidence-based practice,
- Enhancing access to up-to-date medical knowledge within clinical workflows.

Importantly, not all AI used in clinical care serves the same function or carries the same risk. AI systems that **inform clinical decision-making** are distinct from systems that **automate, direct, or execute clinical decisions**. Federal policy should continue to recognize and preserve this distinction.

3. The Need for Proportionate Evidence Standards

A central issue raised by this RFI—appropriately—is how evidence should be generated and evaluated for AI used in clinical care.

Elsevier encourages HHS to adopt **proportionate, risk-based evidence expectations** that align with an AI system's intended use and level of clinical impact. In particular:

- AI tools that **retrieve, summarize, or contextualize existing medical knowledge** should not be held to the same evidentiary standards as tools that diagnose, predict, or prescribe.
- Evidence frameworks should recognize the value of **real-world evidence**, post-market performance monitoring, and continuous improvement—especially for AI systems that evolve over time. Overly rigid or one-size-fits-all evidence requirements risk slowing adoption of beneficial tools without commensurate gains in patient safety.

For CDS tools such as ClinicalKey AI, trust is built through **transparent sourcing, traceability to underlying evidence, clinician reviewability, and clear limitations**, rather than through automation or prescriptive outputs.

4. Regulatory Clarity and CDS Boundaries

Regulatory uncertainty remains a significant barrier to responsible AI adoption in healthcare.

Elsevier supports continued efforts by FDA, ONC, and HHS to:

- Provide **clear, consistent guidance** on the boundary between non-device CDS and regulated Software as a Medical Device (SaMD),

- Reinforce the importance of **human oversight**, explainability, and the clinician's role as the final decision-maker,
- Avoid regulatory approaches that inadvertently push low-risk, informational AI tools into medical device frameworks solely due to the use of advanced AI techniques.

Clarity in this area enables healthcare organizations to adopt AI confidently, while allowing developers to invest responsibly in safety, quality, and innovation.

5. Adoption, Trust, and Real-World Deployment

From Elsevier's experience deploying CDS at enterprise scale, successful AI adoption depends on more than technical performance. Key enablers include:

- Integration into existing clinical workflows and health IT ecosystems,
- Alignment with clinical governance, training, and change management,
- Clear communication of intended use, limitations, and appropriate reliance,
- Organizational trust in the provenance and independence of clinical content.

Federal policy can support adoption by reinforcing these principles and avoiding incentives that prioritize speed or novelty over safe, clinically meaningful use.

6. Closing

Elsevier supports HHS's goal of responsible AI adoption in clinical care and appreciates the opportunity to contribute to this dialogue. We believe that policies grounded in **risk-based regulation, proportionate evidence standards, and respect for clinical judgment** will best enable AI to help improve care quality, clinician experience, and patient outcomes.

We welcome continued engagement with HHS, FDA, CMS, and ONC as these policies evolve.

Respectfully submitted,

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